

SECTION 3

Jurisdictional Fracture

The constitutional architecture of Indigenous disability in Canada, the jurisdictional wasteland the Supreme Court named, and why the ADAP transition reproduces the pattern one more time

There is no hierarchy of rights. But there is a hierarchy of access. An Indigenous person with a disability in Canada navigates the distance between the two every time they need the system to function.”

This section documents the structural feature of Canadian federalism most directly responsible for the Indigenous disability access pattern: the jurisdictional fracture between federal and provincial authority, the Indigenous-specific categories that fall across that line, and the sustained practice — recognized by the Supreme Court of Canada in 2016 as a “jurisdictional wasteland” — of federal and provincial governments treating the population in the fracture as the other level’s responsibility.

The section is structured in four parts. First, the constitutional framework: section 91(24) of the Constitution Act 1867, the division of powers that places “Indians, and Lands reserved for the Indians” in federal jurisdiction while placing healthcare, education, child welfare, housing, and social assistance in provincial jurisdiction. Second, the Indigenous-specific categories that live across the fracture: status First Nations persons on reserve, status First Nations persons off reserve, non-status First Nations persons, Métis, and Inuit, each of whom faces a different set of rules. Third, the judicial and political history of the fracture: *Daniels v. Canada* (2016), *Jordan’s Principle*, and the Canadian Human Rights Tribunal’s repeated non-compliance findings. Fourth, the direct application to AISH and the July 1, 2026 ADAP transition: the reproduction of the same pattern in Alberta provincial disability policy, and the identification of Indigenous disabled Albertans as a specific sub-population of the broader “Alberta Disability Refugee” phenomenon documented elsewhere in this report series.

The central finding: the jurisdictional fracture is not a neutral administrative feature of Canadian federalism. It is a structural design whose documented outcome, across six decades of Supreme Court decisions, human rights tribunal orders, parliamentary motions, and federal–provincial transfer agreements, is that Indigenous people with complex needs are systematically placed in the administrative space between jurisdictions — and systematically harmed there.

Part 1: The constitutional architecture

Section 91(24) and the division of powers

The Constitution Act, 1867 — formerly the British North America Act — distributed legislative authority between the federal Parliament and the provincial legislatures. Section 91 enumerates federal powers. Section 92 enumerates provincial powers. Section 91(24) gives the federal Parliament exclusive legislative authority over “Indians, and Lands reserved for the Indians.” Section 92(7) gives the provinces authority over “the Establishment, Maintenance, and Management of Hospitals, Asylums, Charities, and Eleemosynary Institutions.” Section 92(13) gives the provinces authority over “Property and Civil Rights in the Province.” Section 93 gives the provinces authority over education.

The structural consequence is immediate. The population that section 91(24) assigns to federal jurisdiction — First Nations, and after *Daniels v. Canada* (2016), also Métis and non-status Indians — requires the services that sections 92 and 93 assign to provincial jurisdiction: healthcare, hospitals, social services, education, property law. The Constitution created, at the moment of confederation, a population whose identity sat with one level of government and whose day-to-day service needs sat with the other.

This is not an academic observation. It is the structural reality that has produced, and continues to produce, the access outcomes documented throughout this report. Jordan River Anderson died at age five in a Winnipeg hospital in 2005 not because the medical care he needed was unavailable — it was available in his home community in a specialized foster arrangement — but because the federal government and the Manitoba provincial government spent two years arguing about which of them was financially responsible for the home-based portion of his care. Neither moved. Jordan remained in the hospital. He died there. Section 91(24) versus section 92(7) killed him, in administrative terms. The constitutional architecture did not kill him directly — but the practice of two governments using the architecture to avoid responsibility did.

The Constitution created the fracture. Six generations of federal and provincial governments used it.”

The Indian Act and the categories it created

The Indian Act, first enacted in 1876 and since subject to numerous amendments, is the federal legislation that operationalizes section 91(24). The Act defines who is an “Indian” for federal purposes, establishes the reserve system, and prescribes the administrative apparatus — including band membership, status registration, and the federal agencies now consolidated under Indigenous Services Canada — through which federal responsibility is exercised.

The Act has produced, across 150 years, a series of categories with different rights, different service eligibilities, and different jurisdictional rules:

- Registered (status) First Nations persons: individuals entered in the Indian Register maintained by the federal government, entitled to Indian Act benefits, including tax exemptions for on-reserve income and property, and historically the population for which the federal government has most directly assumed service responsibility.

- Non-status First Nations persons: individuals of First Nations ancestry who, for reasons including historical enfranchisement, intermarriage provisions stripped from the Act by Bill C-31 in 1985, or complex lineage questions, are not entered in the Indian Register. Daniels v. Canada (2016) confirmed that this population falls under section 91(24) federal jurisdiction, but they do not receive Indian Act benefits.
- Métis persons: individuals of mixed European and Indigenous ancestry with ties to the historic Métis nation across the Prairie provinces. Daniels v. Canada confirmed Métis inclusion under section 91(24), but Métis persons have never received Indian Act benefits and historically have been administered through provincial frameworks.
- Inuit persons: determined to be “Indians” for section 91(24) purposes by the 1939 Supreme Court decision Re Eskimo, and administered through federal frameworks including the federal government’s agreements with the Inuvialuit, Nunavut, Nunavik, and Nunatsiavut land-claim organizations.

Each of these categories has different rules, different documentation requirements, different benefit structures, and different administrative points of contact. A disabled Indigenous person applying for provincial disability supports in Alberta must first be classified under one of these categories before any substantive assessment of their disability eligibility begins. The classification question itself is frequently contested, frequently requires proof the applicant does not possess, and frequently depends on the documentary inheritance of historical administrative practices the applicant had no hand in creating.

Part 2: Five populations, five sets of rules

The practical operation of jurisdictional fracture in Alberta is documented in the province’s own Income and Employment Supports Training and Health Benefits (IESTHB) Policy Manual. The Manual explicitly sets out the provincial position on First Nations service eligibility — a position that differs by status, by residence location, by treaty area, and by service type. The following subsection quotes and summarizes the Manual’s own language.

Status on reserve: federal responsibility, provincial disclaimer

The Alberta IESTHB Policy Manual states, under the section titled “First Nations People with Registered Status under the Indian Act”: “When a First Nation person is residing on reserve or ordinarily residing on reserve the Income Support Program responsibility lies with the federal government.” The provincial policy position is unambiguous: if the applicant is a status First Nations person ordinarily resident on reserve, Alberta disclaims responsibility. The federal government, through Indigenous Services Canada and the relevant First Nation government, is the service provider.

This applies to Income Support. The AISH program, which serves adults with severe and permanent disabilities, operates on a parallel but related structure. Status First Nations persons

ordinarily resident on reserve do not, as a matter of ordinary administrative practice, become AISH recipients — they are directed to the federally funded social assistance administered by their band. The structural result is that the AISH caseload of approximately 79,290 Albertans does not reflect the distribution of disability prevalence in the Alberta population. Status First Nations persons on reserve are functionally administered elsewhere.

Status off reserve: provincial responsibility with federal health carve-out

The IESTHB Manual continues: “According to the Administrative Reform Agreement, the province has agreed to fund and deliver the Income Support program to Registered Indians under the Indian Act living off-reserve in the Treaty 7 and 8 areas. The province also funds and delivers Income Support to [off-reserve status Indians in] the Treaty 6 area. The federal government continues to provide social assistance to persons on-reserve.”

Health benefits, however, remain federal. The Manual specifies that registered First Nations people do not receive provincial health benefits through the Income Support program “regardless of where they live since this is the responsibility of the federal government, through Health Canada.” The federal Non-Insured Health Benefits (NIHB) program, administered by Indigenous Services Canada, covers prescription drugs, medical transportation, dental, vision, and mental health services for status First Nations and recognized Inuit persons regardless of residence location.

The practical result: a status First Nations person off reserve in Alberta accesses their income support through the province (with its 24% DTC enrollment rate, its deposit-statement accessibility failures, its call-centre restructure, and its defunded advocacy organizations, all documented in Section 17) and their health coverage through the federal NIHB program (with its own eligibility rules, its own documentation requirements, its own administrative processes). These two systems do not share files, do not share adjudication, and do not share accountability. The recipient navigates both simultaneously.

Non-status First Nations persons and Métis: the Daniels population

Before the 2016 Daniels decision, non-status First Nations persons and Métis occupied what the Supreme Court would describe, in para. 13 of its judgment, as a “jurisdictional wasteland.” The federal government’s position, sustained across decades, was that these populations were not covered by section 91(24) and were therefore a provincial responsibility. Provincial governments, in practice, argued the reverse. Neither level of government organized services around their needs. The population received neither the federally-delivered services available to status First Nations nor the culturally-specific provincial services that might have been designed for them.

The Supreme Court, in its unanimous 2016 Daniels decision, declared that “Métis and non-status Indians are ‘Indians’” under section 91(24) of the Constitution Act, 1867. Justice Abella, writing for the Court, noted explicitly that the declaration was intended to end the “jurisdictional

tug-of-war” (para. 15) in which these groups had been “left wondering about where to turn for policy redress” and to provide “certainty and accountability” for a population that had experienced “denial of material benefits, significant disadvantage and negative consequences.”

The decision is foundational. But it does not, by itself, create federal services where none existed, and it does not compel provincial services to integrate with federal recognition. Ten years after Daniels, the operational reality for non-status First Nations and Métis Albertans seeking disability supports remains structurally similar to the pre-2016 situation: they apply to provincial programs, the provincial programs apply provincial rules, and the federal jurisdiction that Daniels confirmed remains largely theoretical in the context of day-to-day social assistance and disability benefit administration.

The Supreme Court ended the jurisdictional tug-of-war as a matter of constitutional law. It did not end the jurisdictional tug-of-war as a matter of who picks up the phone when a Métis AISH applicant needs a physician to complete a DTC form.”

Inuit persons: federal recognition, service geography

Inuit persons in Alberta are a smaller population, but the structural issues are not different. Federal recognition through the 1939 Re Eskimo decision, federal service delivery through Indigenous Services Canada and federal health coverage through NIHB, and provincial administration of provincially-delivered benefits (housing, AISH, FSCD, PDD) when the Inuit person resides in Alberta rather than in one of the four Inuit Nunangat regions. The Inuit person in Edmonton applying for AISH faces the same multi-jurisdictional adjudication challenge the status-off-reserve First Nations person faces, with the additional complication that the specific federal agencies most oriented to Inuit service delivery are geographically based elsewhere.

Part 3: Jordan’s Principle — the fracture’s most documented case

Jordan River Anderson

Jordan River Anderson was born in 1999 into Norway House Cree Nation in northern Manitoba. He had Carey-Fineman-Ziter syndrome, a rare neuromuscular disorder requiring extensive medical care. He was hospitalized from birth. In 2001, when Jordan was two years old, his medical team determined that his needs could be met in a specialized home setting in or near Winnipeg, with in-home care. The federal government (responsible for services to First Nations on reserve) and the Manitoba provincial government (responsible for services in the province off reserve) could not agree on which would fund the home-based care.

For more than two years, while the two governments continued to dispute financial responsibility, Jordan remained in the hospital. He was not discharged because the placement could not be arranged. The placement could not be arranged because the funding was not

agreed. The funding was not agreed because two governments were in jurisdictional dispute over which of them owed the financial obligation to a child who needed care neither of them was providing.

Jordan River Anderson died in the hospital in 2005 at age five. He never lived in a home. He had been born into a jurisdictional dispute, and he died inside it.

The principle and its promise

Jordan's death catalyzed First Nations advocacy and public awareness of the structural pattern his short life had exposed. On December 12, 2007, the House of Commons of Canada passed Private Member's Motion 296 unanimously. The motion, known thereafter as Jordan's Principle, stated that the federal government should adopt a child-first principle to resolve jurisdictional disputes involving the care of First Nations children. The principle's operative rule: when a First Nations child requires a service that is potentially available under more than one jurisdiction, the government of first contact must provide the service, with jurisdictional responsibility to be resolved between the governments afterward. The child is not to remain in the waiting room while the governments argue.

The motion passed with all-party support. The federal government announced initial funding of \$11 million. According to the 2016 Alanis Obomsawin documentary **We Can't Make the Same Mistake Twice**, the \$11 million initial fund was "never used" — a finding that itself requires pause. A parliamentary motion passed unanimously. A fund was allocated. The fund went unused. The principle was not implemented.

The Canadian Human Rights Tribunal proceedings

In 2007, the First Nations Child and Family Caring Society (led by Dr. Cindy Blackstock) and the Assembly of First Nations jointly filed a complaint with the Canadian Human Rights Tribunal alleging that Canada's underfunding of on-reserve child and family services, and its failure to implement Jordan's Principle, constituted systemic racial discrimination against First Nations children. The federal government contested the complaint for nine years. The Tribunal issued its substantive decision in January 2016, finding that the Government of Canada's implementation of Jordan's Principle was discriminatory and that the federal definition of the Principle had been "narrow" and improper.

The Tribunal ordered Canada to "cease applying its narrow definition of Jordan's Principle and to take measures to immediately implement the full meaning and scope of Jordan's Principle." The Tribunal has subsequently issued a sequence of non-compliance orders against Canada, documenting repeated failures to implement the 2016 decision in full. The 2017 rulings (2017 CHRT 14 and 2017 CHRT 35) found Canada had continued "its pattern of conduct and narrow focus with respect to Jordan's Principle." The Tribunal has, since 2016, specified and re-specified that Jordan's Principle applies to all First Nations children, that service must be based

on the child's needs rather than typical availability, and that administrative procedures must not delay service provision.

By late 2024, the documented backlog of unresolved Jordan's Principle requests had grown to approximately 129,000 cases, including 17,000 deemed urgent. Between July 2016 and September 2025, the federal government approved more than 10 million products, services, and supports under Jordan's Principle — a figure that demonstrates both the real scale of the Indigenous child service need and the scale of the backlog of what has not been approved.

The broader legal framework has produced settlement agreements of extraordinary size: a \$23.4 billion compensation agreement for First Nations children harmed by the on-reserve child and family services system, and a proposed \$47.8 billion long-term reform agreement. These figures are themselves evidence of the scale of the harm. Governments do not pay \$70+ billion in combined compensation and reform unless the harm they are responding to is massive, sustained, and indefensible.

Jordan's Principle as a case study in fracture-response-fracture

The structural pattern that Jordan's Principle was designed to resolve is documented in the sequence that produced it:

- Fracture: federal–provincial jurisdictional dispute over service responsibility.
- Death: a child dies in hospital while governments argue.
- Response: unanimous parliamentary motion, funding allocation.
- Fracture again: federal government implements the motion narrowly, fund goes unused, Principle functionally not applied.
- Tribunal response: CHRT 2016 finding of discrimination, sequence of non-compliance orders through 2025.
- Fracture again: 129,000 pending requests, 17,000 urgent, settlement reform continues to be contested, Principle continues to be implemented in tension with its own judicial elaboration.

At no point in this sequence has the underlying constitutional architecture — section 91(24) against sections 92 and 93 — been altered. The fracture that killed Jordan River Anderson in 2005 remains the fracture through which 129,000 requests are backlogged in 2024. The tribunal orders, the parliamentary motion, the multi-billion-dollar settlements, and the child-first principle itself have not closed the fracture. They have responded to it. The fracture has continued to produce the pattern that prompted the responses.

Part 4: The fracture and the ADAP transition

The Alberta Disability Refugee, Indigenous dimension

The Alberta Disability System Breakdown report series has documented, across nine provincial comparison reports and an extended Charter/Federal Jurisdiction analysis, the phenomenon of the “Alberta Disability Refugee”: disabled Albertans who would leave the province in response to the ADAP transition, the CDB clawback, the removal of appeal rights, and the absence of rent caps, but who cannot leave because the structural design of provincial disability income makes departure financially impossible. The term is not hyperbole. It describes a population functionally trapped by the interaction between Charter mobility rights under section 6 of the Canadian Charter of Rights and Freedoms, and a provincial program that does not transfer, does not bridge, and does not integrate with receiving-province disability programs.

For Indigenous disabled Albertans, the Alberta Disability Refugee phenomenon operates in addition to, and compounded by, the jurisdictional fracture documented in Parts 1–3 of this section. An Indigenous AISH recipient considering leaving Alberta in response to ADAP faces:

- The general structural cage: no bridge payments, no formal interprovincial transfer protocol, no grandfathered eligibility recognition, no relocation assistance.
- Additional Indigenous-specific dimensions: a receiving province that may administer First Nations services differently, a federal NIHB relationship that may or may not transfer seamlessly, a band or community relationship that may not be available in the receiving jurisdiction, a family and cultural support network geographically situated where they currently are.
- The federal-provincial fracture reproduced inside the move: the receiving province’s disability program will apply provincial rules, the receiving province’s First Nations administrative framework will apply federal rules, and the integration between the two in the new province may or may not be better than it was in Alberta.

The result is a population for whom the Charter right to mobility, already rendered functionally void by provincial program design for non-Indigenous AISH recipients, is voided twice over: once by the financial structure of the program, and once by the jurisdictional complexity the move produces.

Why ADAP reproduces the fracture, not resolves it

The Alberta Disability Assistance Program framework that takes effect July 1, 2026 contains no Indigenous-specific provisions. There is no carve-out for recipients whose service access is complicated by the federal–provincial fracture. There is no protocol for integration with Indigenous Services Canada, with the Non-Insured Health Benefits program, or with First Nations band governments. There is no recognition that the Medical Review Panel reassessment process, which will adjudicate which transitioned recipients remain eligible for AISH and which are consigned to ADAP at the reduced benefit level, is a process that operates on standards and documentary expectations more easily met by applicants with stable access

to the professional and documentary infrastructure that federal–provincial fracture historically denies Indigenous applicants.

This is not a failure specific to ADAP. It is a structural feature of provincial disability benefit design across Canada. But it matters specifically here because the July 1, 2026 transition will reassess approximately 79,290 current AISH recipients, and because the population most likely to be reclassified as “employment-ready” under ADAP’s employment-readiness premise is the population least able to contest the reclassification administratively. Indigenous recipients with complicated documentation, with no individual caseworker relationship (the caseworker model was eliminated province-wide in 2025), with limited access to physicians willing and able to complete the paperwork the Medical Review Panel will require, are over-represented in the population most vulnerable to this reclassification.

The ADAP transition does not create this vulnerability. The jurisdictional fracture, documented across 150 years of federal-provincial Indigenous policy, created it. ADAP will, on its current design, reproduce it.

What the Indigenous-specific reading of ADAP adds to the record

Sections 1 and 2 of this report establish the mortality and historical context. Section 17 establishes the federal–provincial clawback pattern for disability recognition instruments — DTC, RDSP, CDB, VISP. Section 3, read in conjunction with 17, establishes that the clawback pattern Section 17 documented for the non-Indigenous disabled Albertan population operates against a baseline of pre-existing Indigenous-specific jurisdictional fracture that has been producing the same kinds of outcomes for the Indigenous disabled population across every federal and provincial government for 150 years.

The non-Indigenous AISH recipient being reclassified under ADAP encounters, for the first time, the kind of administrative filtering that the Indigenous AISH recipient has encountered across every one of the federal and provincial programs they have ever needed. The Alberta Disability Refugee is, in that sense, the universalized version of a status-specific experience Indigenous disabled Albertans have had always. The ADAP transition is not a new kind of harm. It is a familiar kind of harm being newly extended to populations that previously did not experience it.

Indigenous disabled Albertans are not a separate case in the ADAP transition. They are the documented precedent. Everything the rest of the AISH community is about to experience, they have already experienced.”

What this section establishes for the remainder of the report

The constitutional architecture places Indigenous people in federal jurisdiction and the services they need in provincial jurisdiction. The 150-year operational practice of both levels of government has been to use the fracture to disclaim responsibility. Jordan’s Principle, the

Daniels decision, the Canadian Human Rights Tribunal proceedings, and the \$70+ billion in combined compensation and reform commitments are the accumulated cost of that practice — paid, belatedly, by the federal government for harms it could have prevented by consistently acknowledging federal jurisdiction in the first place.

The ADAP transition reproduces the fracture one more time. Not by invoking section 91(24) as a disclaimer — ADAP does not even reach that level of jurisdictional acknowledgment — but by administering provincial disability benefits on the assumption that a standardized reassessment process applies equally to populations whose service relationships with the state are structurally different and structurally disadvantaged.

Sections 4 through 15 of this report document specific manifestations of that structural disadvantage: housing, water, healthcare, Jordan's Principle in child services, child welfare, justice, missing and murdered women, consultation failure, Sovereignty Act conflict, TRC Calls to Action. Each is a concrete application of the constitutional architecture this section has described. Each is a documented pattern. None has been resolved. All continue into the July 1 transition.

This is the structural context into which ADAP is being introduced. Not a vacuum. Not a newly-discovered administrative challenge. The oldest pattern in Canadian federalism, reproduced one more time, at scale, with deadline.

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